

CLINICAL INTELLIGENCE TREND REPORT

YELLOW: Closer Observation Suggested

Patient ID: Thornburg

Period: April 1 to April 30, 2026

Report Date: May 1, 2026

Coverage: 293/720h (41%)

Decision-support summary derived from longitudinal vital sign trends; interpret in clinical context.

Patient clinical status over 30 days from Apr 01 to Apr 30

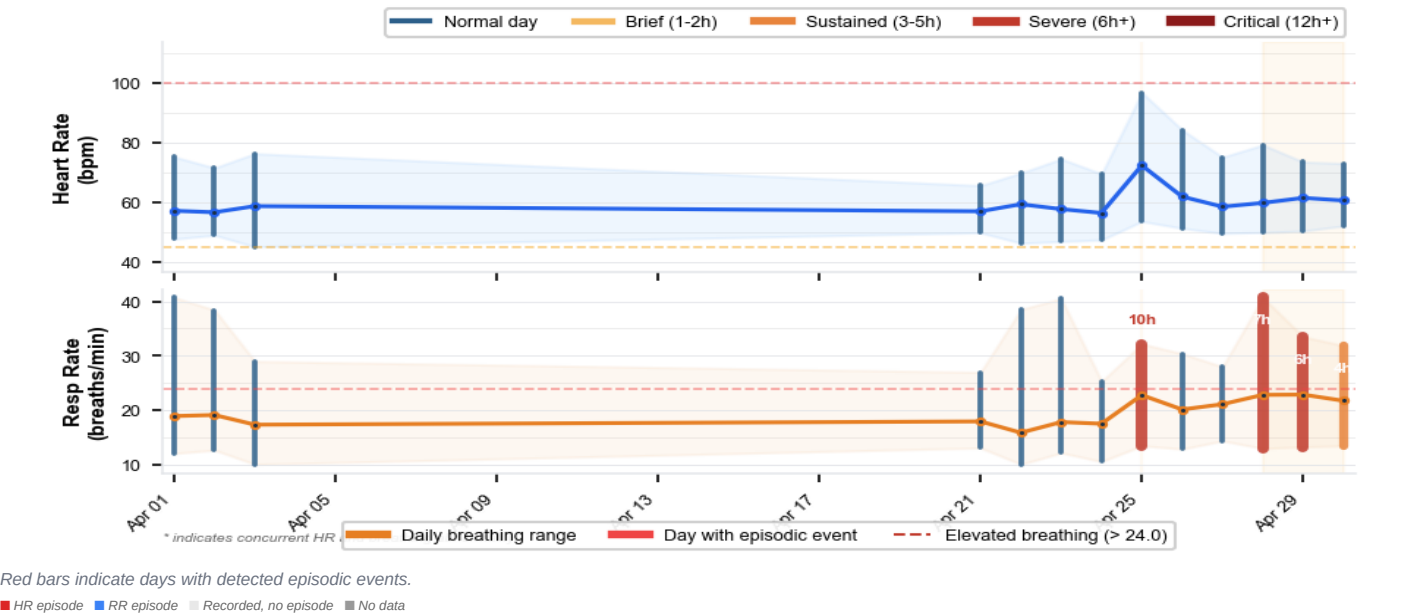


Major Findings: Sustained elevated breathing (avg 25, peak 41)

#	Episode	Min/Max	Total Hours	Longest Continuous	Episodes/d ay	Average	Date
1	Elevated Breathing	32 brpm	11h	6h	3	25 brpm	Apr 25
2	Elevated Breathing	33 brpm	16h	4h	2	25 brpm	Apr 28 to Apr 30

Low data coverage (41%); interpret with caution.

- Clinical Pattern Observations:
- 1. **Sustained finding:** 6h continuous Elevated Breathing on Apr 25.
 - 2. **Clustered pattern:** Episodic events on 4 of 30 days (13%).



Clinical Alerting Thresholds

Very Low HR < 40 bpm

Low HR < 45 bpm

Elevated HR > 95 bpm

High HR > 100 bpm

Very High HR > 110 bpm

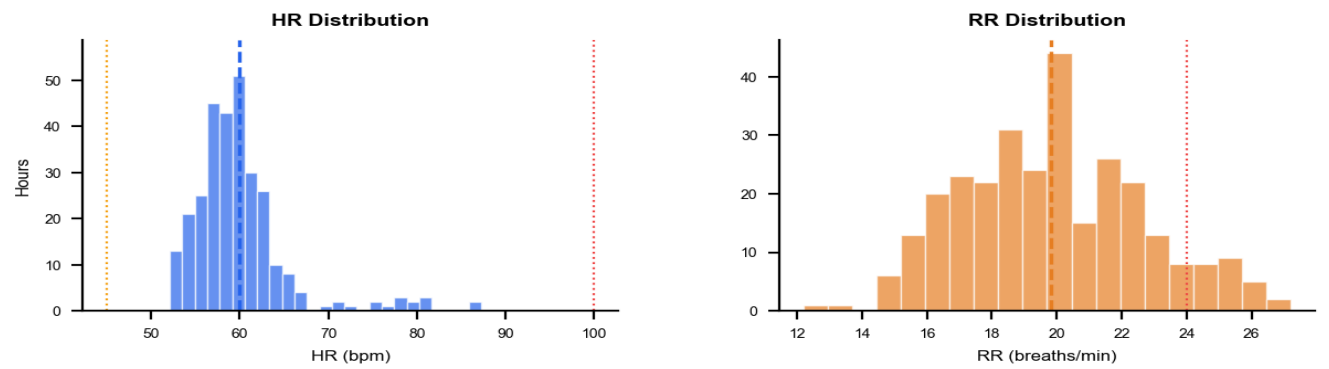
Elevated Breathing > 24 brpm

High Breathing > 30 brpm

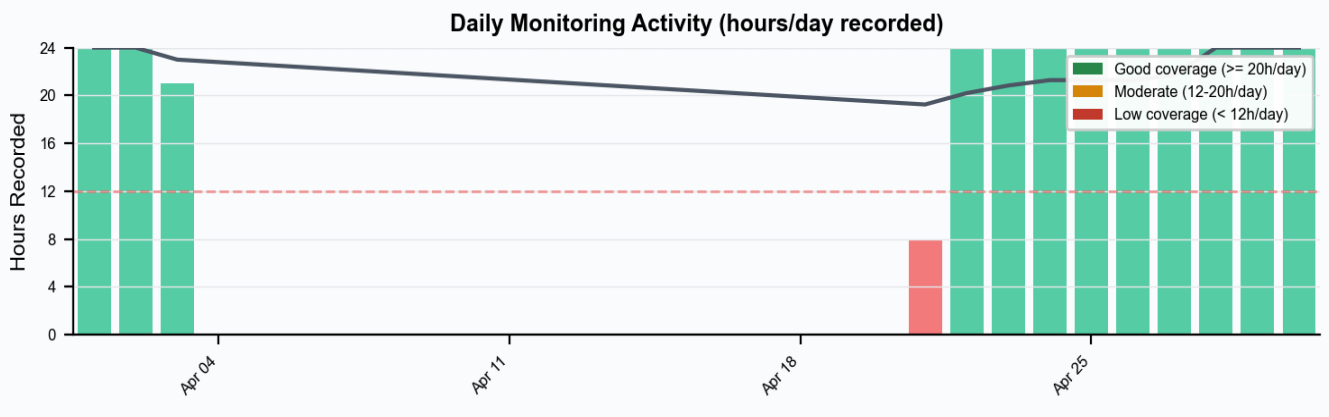
Very High Breathing > 40 brpm

Episodic event: threshold exceeded continuously for ≥ 1 hour. Episodes separated by ≤ 1 hour of normal vitals are counted as the same event. Brief threshold crossings that do not sustain are not flagged.

Vital Sign Distribution



Monitoring Activity



Bars indicate monitoring hours/day. Red threshold line indicates clinical monitoring baseline.